



G. PRASAD

Age: 49 years | Male

Help G. Prasad Undergo Life-Saving Surgery

Your Support Can Give Him a Brighter Tomorrow

Pallavi Geddada



UPI ID: pallaviprasad0416@okaxis

My Father's Story

I am raising this fundraiser for my father, **G. Prasad**, who is suffering from **Crohn's disease** with multiple intestinal strictures and active intestinal inflammation.

His condition has become increasingly difficult to manage. He is currently unable to tolerate solid food. Whenever he tries to eat, he experiences severe cramping abdominal pain and discomfort around his navel. Because he has not been able to eat normally, he has become very weak and has been losing a significant amount of weight in a short period.

His doctors have advised surgical treatment. His hospital documents indicate a planned diagnostic laparoscopy and bowel resection, with or without a stoma, depending on the findings during surgery and what is medically necessary.

Recent CT findings show inflammation involving the small-bowel loops, a possible sinus tract involving the proximal ileum, and a stricture involving the distal jejunum. The doctors have explained that the exact treatment required will depend on the condition of his intestine during surgery. Additional investigations and treatment may also be required.

HOSPITAL ESTIMATE

(AIG Hospitals, Banjara Hills)

Item Name	Amount (INR)
Room Rent (Sharing – 7 days)	24,750
Surgeon Fee	1,70,000
Investigation Charges	As per actual
Drug Charges	1,50,000
Consumable Charges	20,000
Admission Charges	1,200
ICU Charge (2 days)	24,000
Miscellaneous	30,000
Total (Initial Estimate)	4,19,950

Note:

- Blood, tests and other complications are additional.
- The final bill may be higher based on actual treatment, investigations, medicines, ICU requirement and any additional medical needs.
- Expected hospital stay: 7 days
- Minimum deposit: ₹ 3,20,000

Fundraising Goal

₹5,50,000

to cover surgery, hospitalization, investigations, medicines, **blood** requirements, consumables, ICU care if required, and other additional medical expenses.



How Your Support Helps



Surgery
(Planned diagnostic laparoscopy and bowel resection)



Blood requirements
(if needed)



Hospitalization
and room charges



ICU care
(if required)



Investigations
and additional tests



Management of complications



Medicines
and consumables



Other medical expenses

Our family is doing everything possible to arrange the funds, but this cost is beyond what we can manage alone.

Any contribution, no matter how small, can help us get closer to our goal.

If you are unable to donate, please consider sharing this fundraiser with your friends, relatives, colleagues and social networks.

Please keep my father, G. Prasad, in your prayers during this difficult time.

Thank you for your kindness, support and prayers.



Your Support
Makes a Difference

Together
We Can Bring Hope

Thank You
for Standing with Us





Pallavi Geddada



UPI ID: pallaviprasad0416@okaxis

Scan to pay with any UPI app

Surgery Estimation

Patient's UHID/ IPNO	: AIGG.21293071	Patient's Name/Age/Sex	: Mr. PRASAD G/49
Estimation Amount	: 419950		: Year(s)/Male/9700024813
Estimation Date/Time	: 18/09/2026 11:11:32	Estimation Given by	: RAHUL RAO SHINDE
Requested Bed Type	: Sharing	Billing Counselling done by	: Bharath
Surgeon/Physician Name	: Dr. Pradeep Rebala	Billing Counselling done To	: Mr Prasad
Expected day of stay	: 7	Relationship	: Self
Minimum Deposit	: 320000	Emergency No	:
Company	:	Expected Date of Admission	: 18/09/2026
		Estimate Type	: Initial Pre-Admission Estimate
Diagnosis/Medical Problem	: Diag Lap - Small Bowel RA +/- Stoma	Estimate No.	: ESTB1862

Estimation Break-Up-

Item Name	Amount
Room Rent Sharing (5X4950)	24750
Surgeon Fee	170000
Investigation Charges	As per Actual
Drug Charges	150,000.00
Consumable Charges	20,000.00
Admission Charges	1200
ICU Charge (2.00X12,000.00)	24000
Miscellaneous	30000
Total Amount	419950

Remarks : BLOOD, TEST & OTHER COMPLICATIONS ARE ADDITIONAL

Note:

Hospital Admission Terms & Conditions

• Estimates & Billing

The admission cost estimate is approximate. Final billing reflects actual treatment and services received. Enhanced treatment and extended stay costs will be communicated timely. It is the responsibility of patient attendants to check the bill everyday at 3rd floor, IP Billing.

• Advance Payment

Full estimated cost is payable at admission. A minimum positive balance of ₹50,000 must be maintained during the stay.

• Payment Responsibility & Regulations

The patient/legal guardian is responsible for full bill settlement. If a nominee/guardian will pay, a signed undertaking is required. PAN card of the patient or attendant is mandatory at admission.

• Payments & Refunds

Payments accepted via cash, card, DD, UPI, or bank transfer. Cheques are not accepted. Refunds up to ₹ 20,000 will be issued in cash; higher amounts via NEFT within 4-7 working days.

LAMA SUMMARY

UHID	AIGG.21293071	IP No.	4003258
Patient Name	Mr. PRASAD G	Age / Gender	49 years / Male
Doctor Name	Dr. Partha Pal	Room No./Bed No.	432A/432A
Admission Date	01-09-2026 07:01 PM	Discharge Date	03-09-2026 08:02 PM (Type: LAMA)
Patient Address	8-3-232/c/18/b venkatagiri yousufguda hyd, 500045, Telangana		

Primary Consultant :

DR.PARTHA PAL
MBBS (GOLD MEDALIST), MD((GOLD MEDALIST)
DNB (GASTRO), MRCP(UK), ESEGH, FASGE,FACG
DIRECTOR, IBD CENTRE
MEDICAL GASTROENTEROLOGY

Referral Consultant :

Dr. Pradeep Rebala
M.S., MCh, FACS, FRCS (Ed), FRCS (Eng)
Consultant Surgical Gastroenterologist

Diagnosis :

ICD	Diagnosis
	CROHN'S DISEASE - A3 L3+L4 B2+B3
	SUBACUTE INTESTINAL OBSTRUCTION (STRICTURE AT DISTAL JEJUNUM AND PROXIMAL ILEUM)

History of Presenting Complaints :

Patient came with complaints of increasing abdominal pain from 3-4 weeks all over the abdomen, increased on food intake. H/o abdominal distension from 2 months, increased on intake of food and fluids. no complaints of fever, abdominal pain, vomiting or hematemesis, blood in stool.

Background:

Patient was diagnosed with Crohn's disease in 2018. He was treated elsewhere with Mesacol twice daily from 2018. Patient had history of Azoran intake which was discontinued later (? cause). Patient was evaluated again in February 2026 for complaints of abdominal pain and abdominal distention. Colonoscopy and biopsy were done. CT enterography was done on 03/03/2026. He was started on Wysolone 20 mg once daily 20 days ago.

LAMA SUMMARY

UHID	AIGG.21293071	IP No.	4003258
Patient Name	Mr. PRASAD G	Age / Gender	49 years / Male

Colonoscopy(24/02/2026): Multiple superficial ulcers with pseudopolyps in caecum ? Crohn's colitis Biopsies taken for HPE Small external hemorrhoids **Microscopy (24/02/2026):** Multiple colonic mucosal fragments with focally ulcerating epithelium and granulation tissue Few fragments showing moderate to marked activity Lamina propria with moderate-marked lymphoplasmacytic infiltrate, along with neutrophils and eosinophils Focal prominence of lamina propria eosinophils Mild focal crypt disarray No significant basal plasmacytosis No granulomas, parasites, dysplasia or malignancy Similar morphology on serial sections Impression: Histological features of chronic active colitis.

CT Abdomen - 03 Mar 2026 Incomplete stricture involving the distal jejunum/proximal ileal loop, with: Mural thickening and edema Patchy mural stratification Mucosal hyperenhancement Significant adjacent fat stranding Increased mesenteric vascularity Resultant proximal small-bowel dilatation in the right iliac/lumbar regions. Few peripherally enhancing nodules with central hypodensity, abutting the involved bowel -- ? necrotic lymphadenopathy / ? entrapped bowel. Terminal ileum, ileocecal junction, caecum and proximal ascending colon show concentric wall thickening with mucosal enhancement. Mesenteric and pericaecal lymphadenopathy. Tiny non-obstructive left renal calculus, 2.6 mm. No free fluid. Overall features are consistent with inflammatory bowel disease with active disease

Condition on Admission :

Temp	Pulse	Systolic BP	Diastolic BP	SPO2	Height	Weight	BMI
96.8	78	110	70	97	.	.	.

General / Physical Examination :

Patient is conscious / coherent

No pallor, no icterus, no clubbing, no cyanosis, no lymphadenopathy, no pedal edema.

Systemic Examination :

Cardiovascular system: S1, S2+

Respiratory System: BAE+

CNS: Conscious, obeying simple commands

Per abdomen: Distended, mild tenderness + all over the abdomen

Course in Hospital :

Patient was admitted with above mentioned complaints. RFT showed (urea/creatinine 24/0.97, sodium/potassium 132/4.9).

LAMA SUMMARY

UHID	AIGG.21293071	IP No.	4003258
Patient Name	Mr. PRASAD G	Age / Gender	49 years / Male

Patient was treated with RT insertion and aspiration and around 350 mL of RT output was present (? food). Surgical Gastroenterology opinion was taken in view of suspected obstruction and advice followed.

CECT abdomen was done on 02/09/2026 which showed known Crohn's disease with active stricturing disease involving the distal jejunum and proximal ileum, causing moderate to severe luminal narrowing and resultant upstream small-bowel dilatation with distal bowel collapse - consistent with high grade partial small bowel obstruction. Associated mild penetrating disease adjacent to the ileal lesion in the right hypochondrium with heterogeneously enhancing incomplete sinus tracts. No associated abscess. Persistent mild mucosal enhancement of the cecum and proximal appendix -Residual inflammatory change. Mildly enlarged jejunal mesenteric lymph nodes - likely reactive. //

In view of significant obstruction, patient was explained in detail about the need regarding surgery and patient's attenders were not willing for the same and wants to get discharged against medical advice.

The patient was treated conservatively with IV fluids, IV antibiotics (zostum, metrogyl) and other supportive medications. Nature of underlying disease, prognosis and complications has been explained to the patient and patient's attenders in detail. So, the patient is being discharged against medical advice (DAMA).

Radiology Impression :

CECT Abdomen (02/09/2026):

Known Crohn's disease with active stricturing disease involving the distal jejunum and proximal ileum, causing moderate to severe luminal narrowing and resultant upstream small-bowel dilatation with distal bowel collapse - consistent with high grade partial small bowel obstruction. Associated mild penetrating disease adjacent to the ileal lesion in the right hypochondrium with heterogeneously enhancing incomplete sinus tracts. No associated abscess. Persistent mild mucosal enhancement of the cecum and proximal appendix -Residual inflammatory change. Mildly enlarged jejunal mesenteric lymph nodes - likely reactive.

Investigations During Hospitalization :

RFT-RENAL FUNCTION TESTS (UREA, CREAT, ELECT)	Ref. Range	03/09/2026
BLOOD UREA.	16.6 - 48.5	24 mg/dL
SERUM CREATININE.	0.7 - 1.2	0.97 mg/dL
SODIUM.	136 - 145	134 mEq/L

LAMA SUMMARY

UHID	AIGG.21293071	IP No.	4003258
Patient Name	Mr. PRASAD G	Age / Gender	49 years / Male

POTASSIUM.	3.5 - 5.1	4.9 mEq/L
CHLORIDE.	98 - 107	100 mEq/L
SERUM ELECTROLYTES	Ref. Range	01/09/2026
SODIUM	136 - 145	132 mEq/L
POTASSIUM	3.5 - 5.1	4.9 mEq/L
CHLORIDE	98 - 107	98 mEq/L

Condition On Discharge :

Temp	Pulse	Systolic BP	Diastolic BP	SPO2	Height	Weight	BMI
98.6	84	102	73	99	.	.	.

Discharge Medications :

SnoName	Route	Frequency	When	Duration	Start-Date	Instructions
1. SOMPRAZ IT - CAP (ESOMEPRazole(40MG)+ITO-PRIDE(150MG))	Oral	Once Daily 1- 0-0	Before Breakfast	1 Month	03/09/2026	
2. BEVON 200ML SYP (MULTI-VITAMINE)	Oral	Twice a Day 1- 0-1	After Food	1 Month	03/09/2026	
3. DROTIN 40MG TAB (DRO-TAVERINE)	Oral	SOS			03/09/2026	

Diet Care :

Exclusive enteral nutrition as advised by dietitian.

Review / Follow-up :

Urgent hospitalization required. Plan exploratory laparotomy and proceed.

Review SOS in Medical Gastroenterology OPD.

VERIFIED BY:

LAMA SUMMARY

UHID	AIGG.21293071	IP No.	4003258
Patient Name	Mr. PRASAD G	Age / Gender	49 years / Male

DR.PARTHA PAL

MBBS (GOLD MEDALIST), MD((GOLD MEDALIST)

DNB (GASTRO), MRCP(UK), ESEGH, FASGE,FACG

DIRECTOR, IBD CENTRE

MEDICAL GASTROENTEROLOGY

for Prasad

LABORATORY INVESTIGATION REPORT

Patient Name : Mr. PRASAD G
UHID : AIGG.21293071
Episode : IP
Ref. Doctor : Dr. Partha Pal

Age/Sex : 49 Year(s) / Male
Order Date : 03/09/2026 10:07
IP/Bed : 4003258 / 432A
Mobile No : 9700024813
Facility : AIG Hospitals, Banjara Hills

BIOCHEMISTRY AND THERAPEUTIC DRUG MONITORING

TEST	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Sample No : ABI0061498A	Collection Date : 03/09/26 10:07	Ack Date : 03/09/2026 11:04	Report Date : 03/09/26 11:53

RFT-RENAL FUNCTION TESTS (UREA, CREAT, ELECT)

Sample Type- Serum

BLOOD UREA. <i>Method - urease UV</i>	24	mg/dL	16.6 - 48.5
SERUM CREATININE. <i>Method - KINETIC JAFFE or alkline picrate</i>	0.97	mg/dL	0.7 - 1.2
SODIUM. <i>Method - ISE</i>	134 ▼ (L)	mEq/L	136 - 145
POTASSIUM. <i>Method - ISE</i>	4.9	mEq/L	3.5 - 5.1
CHLORIDE. <i>Method - ISE</i>	100	mEq/L	98 - 107

Interpretations: Aids in diagnosis and management of conditions affecting kidney function. Useful for general health screening, screening patients at risk of developing kidney disease, management of patients with known kidney disease.

Report Saved By - Firdous sultana (03/09/2026 11:53 AM)

End of Report

Deepika

Dr.G. DEEPIKA

Director & HOD - Biochemistry
MBBS, MD
RegNo: 54289

Name : Mr PRASAD G
UHID : AIGG.21293071
Gender/Age : M/49 y
Ref. Phys. : Doctor Partha Pal
Procedure Name : CT ABDOMEN WITH CONTRAST.

Accession No : 11.2082634.33749
Bill Date : 02-Sep-2026 18:03
Reporting Date : 03/09/2026 12:32PM
Modality : CT

DEPARTMENT OF RADIOLOGY & IMAGING

CECT ABDOMEN

TECHNIQUE: Pre- & post contrast arterial, venous Helical CT acquisition of abdomen was done on 160 slice United Imaging uCT 780CE CT scanner & images were reconstructed in axial & coronal planes.

Clinical details: C/o abdominal pain, distension.
CT (3/3/26) shows incomplete stricture in distal jejunum and proximal ileum.
History of Crohn's disease in 2018, treated.

CT dated 3/3/26 (ID 19978) is available for comparison.

FINDINGS:

Bowel, mesentery, omentum:

Nasogastric tube insitu.

Short segment circumferential enhancing wall thickening involving the proximal ileum in the right hypochondrium causing moderate to severe luminal narrowing resulting in dilatation of proximal bowel loops (maximum luminal diameter 6.5cm in the proximal jejunum) and collapse of the rest of the small bowel loops. The thickened segment shows homogenous enhancement and measures 2.8cm in length with maximum wall thickness of 12mm. There is mild penetrating disease adjacent to the lesion with heterogeneously enhancing incomplete sinus tracts. No evidence of abscess.

Short segment circumferential enhancing wall thickening involving the distal jejunum causing moderate to severe luminal narrowing resulting in dilatation of proximal bowel loops (maximum luminal diameter 3.6cm in the proximal jejunum) and collapse of the rest of the small bowel loops.

The thickened segment measures ~ 4.5cm in length with maximum single wall thickness of 9.8mm and shows homogenous enhancement with no mural stratification / submucosal edema.

Resolution of mucosal thickening seen in the terminal ileum and ileocecal junction.

Persistent mild mucosal enhancement seen in the cecum and proximal body of appendix.

No mesenteric fibrofatty proliferation.

Multiple enteroliths seen in the mid dilated jejunal loops above the stricture.

Rest of the bowel loops are normal. No evidence of malrotation.

Lymph nodes:

Jejunal mesenteric node measures 11.8mm
Ileal mesenteric node measures 5.6mm.

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Gender/Age : M/49 y
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Accession No : 11.2082634.33749
Bill Date : 02-Sep-2026 18:03
Reporting Date : 03/09/2026 12:32PM
Modality : CT

Liver: Normal in size, contour, attenuation and enhancement.
No focal hypo / hyperenhancing lesion.
Tiny calcific granuloma in right lobe.
Hepatic veins and IVC are patent.

Splenoportal axis:

Main portal vein, splenic and superior mesenteric veins are normal in course and calibre. No evidence of thrombosis. No collaterals

Gallbladder and biliary tree: Well distended. Wall thickness appears normal. No radiodense calculi. Common hepatic and common bile ducts are normal. No intrahepatic biliary radicle dilatation.

Spleen: Normal in size, shape, attenuation and enhancement. No focal lesions noted.

Pancreas: Normal in size, shape, attenuation and enhancement. No e/o ductal dilatation, parenchymal calcifications and peripancreatic fluid collection.

Kidneys: Normal in size, shape, attenuation and enhancement. No hydronephrosis / calculi / focal lesions.

Adrenal glands: Normal in size, shape, attenuation and enhancement. No focal lesions.

Aorta: Aorta and its branches are patent.

Urinary bladder: Well distended. No calculi. Wall thickness appears normal.

Prostate and seminal vesicles: Normal.

Abdominal wall: Nil significant.

Inguinal orifices: No hernias.

Free fluid: Nil significant.

Visualized lung bases and pleura: Nil significant.

Visualized bones: Osteopenia of pelvic bones.

IMPRESSION:

Comparison done with CT dated 3/3/26 (ID 19978).

Known Crohn's disease with active stricturing disease involving the distal jejunum and proximal ileum, causing moderate to severe luminal narrowing and resultant upstream small-bowel dilatation with distal bowel collapse - consistent with high grade partial small bowel obstruction.

Name : Mr PRASAD G
UHID : AIGG.21293071
Gender/Age : M/49 y
Ref. Phys. : Doctor Partha Pal
Procedure Name : CT ABDOMEN WITH CONTRAST.

Accession No : 11.2082634.33749
Bill Date : 02-Sep-2026 18:03
Reporting Date : 03/09/2026 12:32PM
Modality : CT

Associated mild penetrating disease adjacent to the ileal lesion in the right hypochondrium with heterogeneously enhancing incomplete sinus tracts. No associated abscess.

Persistent mild mucosal enhancement of the cecum and proximal appendix -*Residual inflammatory change.*

Mildly enlarged jejunal mesenteric lymphnodes - likely reactive.

This examination and reported findings have been reviewed and confirmed.

Approved by:



Dr. Chaithanya Isamalla
MBBS, MDRD
Director - Sr Consultant Radiology

*****End Of Report*****

LABORATORY INVESTIGATION REPORT

Patient Name	: Mr. PRASAD G	Age/Sex	: 49 Year(s) / Male
UHID	: AIGG.21293071	Order Date	: 01/09/2026 21:25
Episode	: IP	IP/Bed	: 4003258 / 432A
Ref. Doctor	: Dr. Partha Pal	Mobile No	: 9700024813
		Facility	: AIG Hospitals, Banjara Hills

BIOCHEMISTRY AND THERAPEUTIC DRUG MONITORING

RESULT

Sample No: ABI0061161A Collection Date: 01/09/26 21:25 Ack Date: 01/09/2026 21:54 Report Date: 01/09/26 22:09

SERUM ELECTROLYTES

Sample Type- Serum

SODIUM	132 ▼ (L)	mEq/L	136 - 145
Method - ISE INDIRECT			
POTASSIUM	4.9	mEq/L	3.5 - 5.1
Method - ISE INDIRECT			
CHLORIDE	98	mEq/L	98 - 107
Method - ISE INDIRECT			

Interpretations: The electrolytes, sodium, potassium and chloride are useful to identify electrolyte and fluid imbalance that are often indicative of many acute and chronic illnesses. Electrolyte concentrations assist in investigating conditions that cause electrolyte imbalances including dehydration, kidney disease, lung diseases and heart conditions. Repeat testing can be used to monitor the imbalance until remedied.

Report Saved By - Salke Anjaneyulu (01/09/2026 22:09 PM)

End of Report



Dr.G. DEEPIKA

Director & HOD - Biochemistry
MBBS, MD
RegNo: 54289